BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Milestones show order, not equal time intervals or booked appointments. The treating team requests tests, reviews results and confirms treatment. full blood count (FBC), renal and liver profile before each 21-day cycle; review infection, neuropathy, oedema, respiratory symptoms and steroid premedication before each infusion. National Cancer Control Programme (NCCP) breast treatment continues until progression or unacceptable toxicity. The source six-cycle maximum applies to non-small-cell lung cancer (NSCLC), not automatically to breast cancer.

Check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Blood / kidney / liver	Baseline FBC, kidney and liver tests	FBC, kidney and liver tests before each 21-day cycle	FBC, kidney and liver tests before each 21-day cycle	Recheck recovery and dose before restarting	Follow unresolved abnormalities
Symptoms / medicines	Review symptoms, medicines and previous treatment	Review effects before each dose	Continue symptom and medicine reviews	Report problems; do not change or restart doses yourself	Agree recovery and next treatment plan
Heart / electrocardiogram (ECG) / echo	Assess cardiac history and new symptoms; this docetaxel monotherapy source has no fixed serial echo/multigated acquisition (heart pumping) scan (MUGA) schedule.	Review symptoms and indicated tests	Continue the risk- and treatment-specific plan	Urgent assessment for chest pain, fainting or breathlessness	Agree follow-up from exposure and results
Response / course	Confirm cancer-specific treatment aim	Repeat 21-day cycles while beneficial and tolerated	Review benefit, symptoms and the agreed scan plan	Reassess if treatment is held or symptoms change	Agree continuation, change or completion
Bone health / other care	Review previous/ongoing treatments and fracture risk	No automatic chemotherapy-cycle dual-energy X-ray absorptiometry (bone density) (DXA)	Keep any separate endocrine/ fracture-risk plan	Review if clinical risks change	Continue separately indicated follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00203 v7 · official regimen · EMA Taxotere product information

THE WHOLE TREATMENT JOURNEY

Docetaxel 75 mg/m² · every 3 weeks: treatment and checks

Source-defined breast, lung, gastric, gynaecological and castration-sensitive prostate cancer (including high-risk nonmetastatic prostate disease)

Docetaxel 75 mg/m² by intravenous (IV) infusion on Day 1 every 21 days. For the source breast indication, continue until progression or unacceptable toxicity; the team reviews benefit and tolerability. The six-cycle source maximum applies to NSCLC.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Treatment plan NCCP 00203	Confirm indication, goal and current prescription	Repeat 21-day cycles while beneficial and tolerated	Continue according to response, tolerance and prescribed course	Team review before restart or dose change	Agree completion or next treatment
Blood / kidney / liver NCCP 00203 · SMPC	Baseline FBC, kidney and liver tests	FBC, kidney and liver tests before each 21-day cycle	FBC, kidney and liver tests before each 21-day cycle	Recheck recovery and dose before restarting	Follow unresolved abnormalities
Symptoms / medicines NCCP 00203 · SMPC	Review symptoms, medicines and previous treatment	Review effects before each dose	Continue symptom and medicine reviews	Report problems; do not change or restart doses yourself	Agree recovery and next treatment plan
Heart / ECG / echo NCCP 00203 · SMPC	Assess cardiac history and new symptoms; this docetaxel monotherapy source has no fixed serial echo/MUGA schedule.	Review symptoms and indicated tests	Continue the risk- and treatment-specific plan	Urgent assessment for chest pain, fainting or breathlessness	Agree follow-up from exposure and results
Response / course NCCP 00203 · SMPC	Confirm cancer-specific treatment aim	Repeat 21-day cycles while beneficial and tolerated	Review benefit, symptoms and the agreed scan plan	Reassess if treatment is held or symptoms change	Agree continuation, change or completion
Bone health / other care NCCP 00203 · SMPC	Review previous/ongoing treatments and fracture risk	No automatic chemotherapy-cycle DXA	Keep any separate endocrine/ fracture-risk plan	Review if clinical risks change	Continue separately indicated follow-up

What each check is for

1

Treatment and safety review

Before each planned treatment or four-week review for olaparib

FBC, renal and liver profile before each 21-day cycle; review infection, neuropathy, oedema, respiratory symptoms and steroid premedication before each infusion.

Ask / **act:** The treating team confirms results and the plan; this map does not authorise a dose.

Sources: NCCP 00203 · SMPC

2

Benefit and continuation

At agreed clinical review points

Docetaxel may slow or shrink advanced breast cancer and help symptoms. The dose is selected to balance benefit and side effects; results from a different dose or another cancer do not predict your outcome.

Ask / **act:** Discuss benefit, toxicity and priorities before continuing or changing treatment.

Sources: NCCP 00203 · RETROSPECTIVE-TRIAL

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.21